Read the claim. Find the study. Keep the limits.

SECOND LOOK / EVIDENCE READING GUIDE

A practical guide to organizing a drug-repurposing claim before a conversation with a qualified professional. Use it with a paper, a news item, or a discussion link. You will leave with a precise question, a source trail and a short list of unresolved points.

Edition 0.3 - 9 October 2026
Assigned editorial author: Karen Pendergrass. Private AI-assisted draft; human editorial approval and qualified independent review pending. Not approved for public or commercial release.

What this guide contains

Keep the source and the claim separate

Compare the question with the study (full guide)

Read the result faithfully (full guide)

Build a small evidence map (full guide)

Complete your claim worksheet (full guide)

Prepare a focused conversation (full guide)

Sources, limitations and edition record

This is a reading process, not a treatment protocol. It does not rank drugs, tell you whether to take a medicine, or establish the benefit or safety of a particular use. The worked example is deliberately fictional and is not part of the research library.

Keep the source and the claim separate

Begin by saving the exact claim in your own notes and recording where you saw it. Then write a neutral question. Replace “Does this drug work?” with “For which people, for which condition, using which formulation and route, compared with what, and measured by which outcome?” Keep prevention and treatment as separate questions.

Find the report behind the headline

An abstract is a short account of a report; the main paper contains separate methods, results, discussion and reference sections.[1] Record which parts you could actually read. If you have only an abstract, write “abstract only” and list the details that still need checking. A discussion link belongs in the question trail; put the actual study in the evidence trail.

Basic research, clinical trials and reviews serve different purposes; a review brings together other studies rather than becoming another original trial.[2] Add a source-type label before copying a conclusion into your notes. Keep a trial registry record, a paper and a later follow-up linked when they describe the same investigation.

Check the use, not only the drug name

In the United States, off-label use can involve a different condition, route or dose. FDA approval for one use does not establish FDA approval or a favorable FDA assessment for every proposed use.[3] Record the jurisdiction and the exact current official source; leave status unresolved when that check has not been completed.

Sources, limitations and edition record

Checked 8 October 2026. These are methodological/regulatory CONTEXT sources, not original drug trials. No systematic review or drug-specific appraisal is claimed.

[1] NCCIH: Abstract and Main Sections
Abstract; Main Sections.

[2] NCCIH: Types of Research
Methods > Types of Research.

[3] FDA: Understanding Unapproved Use of Approved Drugs
What are examples of unapproved uses of approved drugs?; Questions you may want to consider.

[4] NCCIH: Design of the Study
Methods > Design of the Study; Study A and B.

[5] NCCIH: Minimizing Bias
Methods > Minimizing Bias.

[6] NCCIH: Results
Results > clinical significance, replication and conflicts.

Review and release

Karen Pendergrass - Assigned editorial author. AI-assisted draft; assignment is not independent scientific approval

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Independent qualified reviewer: not assigned. Human editorial approval: pending. Five-reader comprehension/source-finding exercise: not run. Public and commercial release: HOLD. The fictional arithmetic exercise is excluded from the evidence corpus.

Edition 0.4 uses the shared renderer; evidence-reading content is unchanged. Corrections should identify the edition, page, sentence and source. Changed editions require renewed review.

Human editorial approval and qualified independent review are pending; reader testing has not run. Public and commercial release: HOLD.

Corrections and edition updates | Educational-use policy